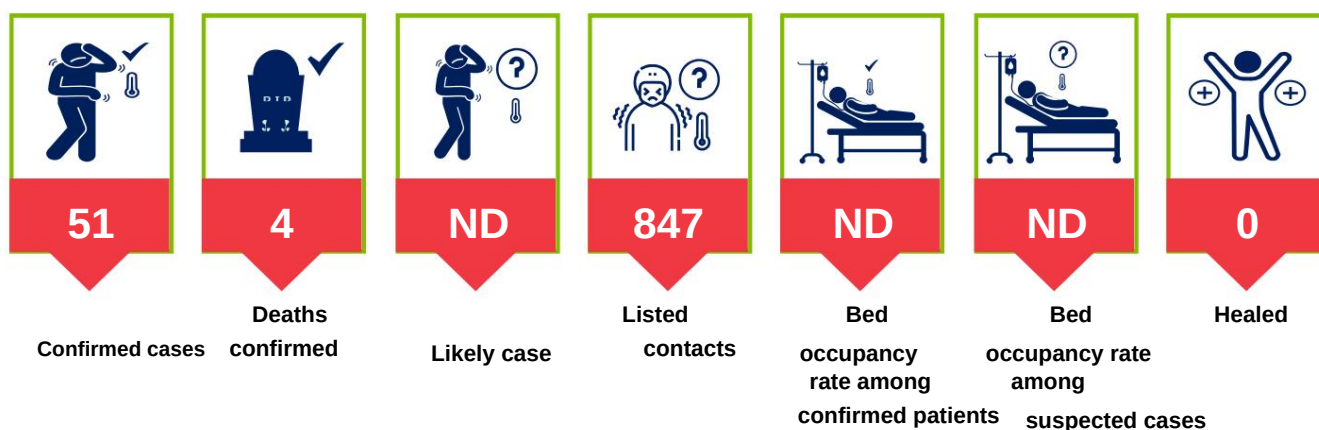


Public Health Emergency Operations Centre "COUSP-DRC"

Situation Report of the 17th Ebola Virus Disease Outbreak/DRC

SitRep MVE No. 005/MVE_17/2026

Affected provinces	ITURI, NORTH KIVU
Affected Health Zones	ITURI : Mongbwalu, Rwampara, Bunia, Nyankunde, NORTH KIVU : Goma, Katwa and Butembo
Reporting date	May 19, 2026
Publication date	May 20, 2026



Affected provinces	Health zones	New case confirmed	New deaths confirmed	New case probable	New case suspects	New contacts listed	Healed by the day
Ituri	Bunia	0	0	ND	29	76	0
	Rwampara	4	0	ND	18	215	0
	Mungbwalu	12	0	ND	7	15	0
	Nyankunde	0	0	ND	5	0	0
Subtotal		16	0	0	59	306	0
North Kivu	Goma	0	0	ND	ND	ND	0
	Butembo	0	0	ND	ND	ND	0
	Katwa	0	0	ND	ND	ND	0
Subtotal		0	0	0	0	0	0
Total		16	0	0	59	306	0



High-level meeting held
by the Minister of
Communication with the
Minister of Health, the Director
General of Africa CDC, Muyembe
and the Director General of INSP

I. CONTEXT

The Ituri province is located in the northeast of the Democratic Republic of Congo and shares a long border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. It has an estimated population of over 8 million, including more than one million internally displaced persons.

In September 2018, this province was affected by an Ebola virus disease epidemic that was raging in the neighbouring province of North Kivu.

The Mongbwalu health zone is one of the 36 health zones in Ituri province. Considered the starting point of the epidemic, it is located in the Djugu territory, where several armed groups operate. It is situated 70 km from Bunia, the capital of Ituri province, with frequent population movements towards Uganda.

This locality has the distinction of having several construction sites which attract people from other areas of the province and neighboring provinces.

II. HISTORY OF THE EPIDEMIC

III.

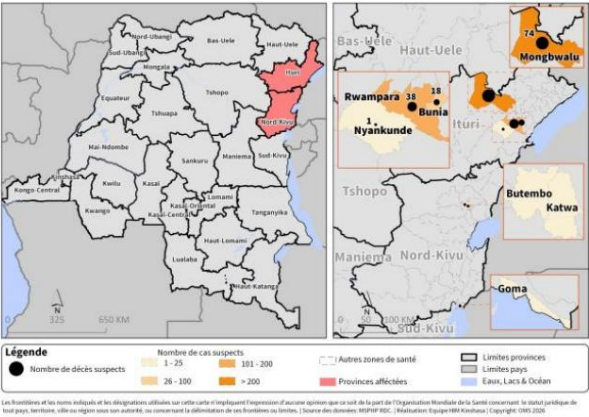
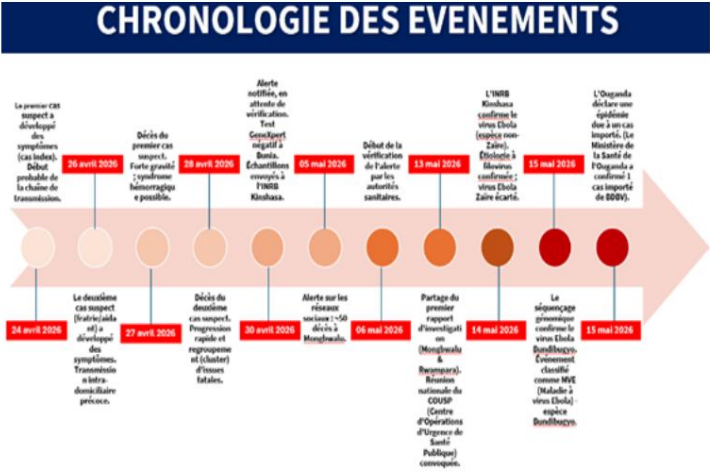
On May 5, 2026, social media reported a surge in deaths of unknown cause, with nearly 50 deaths already recorded in the Mongbwalu health zone. Following confirmation of this alert, the Chief Medical Officer of the Mongbwalu health zone shared the preliminary investigation report on May 9, 2026. On May 11, 2026, provincial health authorities held an emergency meeting and decided to deploy rapid response teams to Mongbwalu and Rwampara, which had reported suspected deaths from viral hemorrhagic fever among patients hospitalized at the Rwampara Evangelical Medical Center, who had been transferred from Mongbwalu.

Investigations conducted on May 12, 2026, simultaneously in the Mongbwalu and Rwampara health zones by the Rapid Intervention Teams of the Ituri Provincial Health Division (DPS), revealed that most suspected patients or deaths presented with fever, headache, vomiting, and severe physical asthenia. Deaths occurred 2 to 3 days after the onset of illness, with symptoms including hematemesis, epistaxis, or blackish vomit. During the investigations in the Rwampara health zone, 20 samples were collected from suspected cases or deaths. The majority of samples were taken from the Mongbwalu health zone. Preliminary tests conducted at the Bunia Laboratory were all negative for Ebola Zaire, dengue fever, rotavirus, cholera, malaria, Yersinia pestis, measles, and COVID-19. These samples were sent to the National Institute of Biomedical Research (INRB) in Kinshasa for further testing.

As of May 14, 2026, **13 of these 20 samples were tested, and 8 were found to be positive** for non-Zairean Ebola virus disease at the INRB in Kinshasa. On May 15, 2026, sequencing of the positive samples identified the BUNDIBUGYO strain, *allowing the Minister of Public Health, Hygiene and Social Welfare (MSHPS) to officially declare the 17th Ebola virus disease outbreak.*

Ebola virus disease (Bundibugyo strain), thus marking the 17th epidemic in the country's history

As of May 19, 2026, a total of 51 cases have been reported in two provinces (49 in Ituri and 2 in North Kivu), with 99 samples analyzed out of the 183 received to date.



II. EPIDEMIOLOGICAL SITUATION UPDATE

- 16 New confirmed cases as of May 19, 2026 in the province of Ituri.
- As of May 19, 2026, there were a cumulative total of 51 confirmed cases, including 48 confirmed cases in Ituri and 3 in North Kivu

Presentation of the situation of the 17th Ebola outbreak in DRC at the IHR global emergency committee meeting.

Table I. Summary of the alert status as of May 19, 2026

Zones de santé	Report des alertes (18/05)	Nouvelles alertes (19/05)	Total des alertes du jour (19/05)	Alertes vivants	Alertes décédées	Alertes investiguées	Alertes non investiguées	Alertes validées	Alertes invalidées	Cas suspects prélevés	Cas suspects non prélevés	Cas suspects isolés	Cas suspects non isolés
Bunia	0	35	35	30	5	33	2	29 (5 décès)	4	10	19	30	0
Rwampara	8	19	27	22	5	20	7	18	2	13	5	13	0
Nyankunde	0	6	6	2	4	5	1	5	0	5	0	5	0
Mongbwalu	0	7	7	4	3	7	0	7	0	7	0	4	0
Komanda	0	3	3	2	1	3	0	3	0	2	1 (décès)	2	0
Bambu	0	1	1	0	1	1	0	1	0	0	1 (décès)	0	0
Mangala	0	2	2	2	0	2	0	2	0	1	1 (vivant)	1	1
Total	8	73	81	62	19	71	10	36	6	38	27	55	1

Table II. Summary of contact identification and follow-up as of May 19, 2026

Zones de santé	Suivi journalier						Taux de suivi	Total non vus	Raison de non vus						Reste à suivre le jour suivant
	Report du 18/05/2026	Nouveaux contacts le 19/05/2026	Contact à suivre ce jour	Contacts vus	Sorties 21 Jrs	Devenus Suspects			Simple Absence	Perdus de vue (≥72 heures)	Jamais vus	Refus	Déplacés	Reco n'est pas passé	
Bunia	127	76	203	3	0	1	1,5%	200	0	0	0	0	0	200	203
Rwampara	120	215	335	78	0	0	23%	257	0	0	0	0	0	257	335
Nyankunde	133	0	133	0	0	0	0%	133	0	0	0	0	0	133	133
Mongbwalu	161	15	176	0	0	0	0%	176	0	0	0	0	0	176	176
Total	541	306	847	81	0	1	9,6%	766	0	0	0	0	0	766	847

Table III. Distribution of cases, suspected and confirmed deaths of Ebola virus disease in the Health zones of the DPS Ituri and North Kivu as of May 19, 2026

Provinces	Health zones	Number of cases suspects	Number of deaths suspects	Number of cases Confirmed	Number of contacts
Ituri	Mongbwalu	309	77	13	176
	Rwampara	154	43	25	335
	Bunia	90	23	6	203
	Nyankunde	19	5	4	133
North Kivu	Goma		0		N
	Katwa		0		ND
	Butembo	1 1 1	0	1 1 1	ND
Total		575	148	51	847

*This data is subject to change after harmonization of data at different levels of the health pyramid with the incident management system.

ND= Not available

III. RESPONSE ACTIONS

COORDINATION

- Holding the coordination meeting with the sections.
- Participation in strategic meetings at the governorate and central levels
- Holding coordination meetings with the sections.

Participating in strategic meetings at the governorate and provincial levels. • Sharing priority activities and budgets with the central level. • Establishing and revitalizing intervention teams. • Identifying potential sites for CTE installations.

EPIDEMIOLOGICAL SURVEILLANCE

- Mapping of confirmed cases: Bunia and Rwampara health zones
- Active search for Ebola cases: Bunia, Bambu, Rwampara, Nyankunde health zones • Investigation of Ebola alerts: Bunia, Mongbwalu, Rwampara, Nyankunde, Kamanda, Mangala health zones Bambu and Aru
- **Ofoo /ZS ARU**: originating from Bukavu via Kigali, Uganda, heading for DURBA.
- **PoE Kasenyi/ZS TCHOMIA**: A lifeless body from Uganda/Kampala to the city of Bunia.

LABORATORY

Results as of May 18, 2026 (RADIONE)

☐ Samples reported: 84
☐ Samples received: 46 (New suspected cases) Samples to
be analyzed: 130 Samples analyzed
today: 31 Re-sampling samples
☐ analyzed: 0 Confirmed cases today: 16 (Mongbwalu:
12 and 4 without records) Cumulative confirmed cases: 51 o Ituri
☐ Laboratory: 48 o Goma
Laboratory: 1 o Butembo
Laboratory: 2

RISK COMMUNICATION AND COMMUNITY ENGAGEMENT (RCCE)

- Dissemination of a message from the Military Governor of the Province regarding compliance with measures to combat the Ebola in Ituri Province through various communication channels (radio, schools, social media) • Advocacy interviews with both the Provincial Government Spokesperson and Head of Communications from the Governorate as well as the Head of State Protocol Service for the area.
- Five (5) Advocacy Interviews: with the Provincial Head of EPST Eastern Province 1, Head of Youth Division and Provincial Head of Gender, Family and Child Division, FARDC Commander in the MANGALA Health Zone, with the Provincial Committee of Traditional Practitioners of Ituri; • Successively to prohibit students from being expelled from schools for not wearing masks, for the mobilization of youth organizations in communication activities in support of mobilizing women's associations in community engagement for the success of this response; • Promotion of the local toll-free number (0821419595).

INFECTION PREVENTION AND CONTROL

ZS Bunia

- Implementation of triage at the DPS level, • 1 EDS carried out and decontamination: HGR Bunia, • Briefing of the supervisory and contamination control teams from the ZS/Bunia; DPS supervisors, and LPSP surface technicians.
- Briefing of 3 cleaning staff from HGR-Bunia assigned to the entrance for triage, • Provision of handwashing facilities to the IPC pillar by MUSACA, • Donation of 30 kg of chlorine by Save the Children to the IPC pillar

ZS Rwampara

- Supporting healthcare providers in implementing CS SHARI hand hygiene practices, DELE and La Victoire Health Post,
- Decontamination of the Clinique la GRACE DIVINE and CM GLORIA

ZS Mongbwalu

- Briefing of 3 CTE Mongwalu service providers on standard precautions, • Decontamination of isolation unit, • **GAP:** • Insufficient number of Cleaning Technicians in the Mongwalu General Referral Hospital,
- Lack of training for subcommittee members, • Lack of PPE to carry out EDS.

- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm

III. CHALLENGES

- Strengthen intervention teams in the affected areas,
- Make means of transport (mobility) available to field teams, • Fill the gap in sampling kits,
- Continue compiling contact lists and begin contact tracing in all ZS areas,
- Develop a map of population mobility in the affected health zones,
- Strengthen epidemiological and community-based surveillance, • Train service providers in epidemiological surveillance, community-based surveillance, and laboratory work



Briefing of 52 people, including 25 PCI Supervisors and 27 Cleaning Technicians, on the use of PPE and decontamination procedures.



Decontamination of the ambulance that transported the suspected case



4680 people (768 men, 887 women, 1637 girls, 1389 boys) were reached by volunteers from the DRC Red Cross through door-to-door outreach in the Bigo area.



Session to harmonize activities and draft site plan

**FOR ALL
INFORMATION
ADDITIONAL,
PLEASE CONTACT**

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